

IN THE HIGH COURT OF SOUTH AFRICA
(WESTERN CAPE DIVISION, CAPE TOWN)

Case No: 2026-086906

In the matter between:

AFROCENTRIC INVESTMENT
CORPORATION LIMITED

Applicant

and

WILBUR WILLIAM MATTHEE

Respondent

SUPPLEMENTARY ANSWERING AFFIDAVIT

I, the undersigned, **WILBUR WILLIAM MATTHEE**, do hereby make oath and say:

1. INTRODUCTION AND PURPOSE

- 1.1 I am an adult male Specialist Registered Nurse (NQF Level 8) and Forensic Governance Specialist. The facts contained herein fall within my personal knowledge and are both true and correct.
- 1.2 I file this supplementary affidavit to expose the *mala fides* (bad faith) behind this urgent application. I will demonstrate that this interdict is not a legitimate protection of proprietary data, but rather the culmination of a sustained campaign of **Occupational Detriment** designed to silence a whistleblower and conceal a multi-billion rand statutory financial risk.
- 1.3 I seek specific declaratory findings from this Honourable Court to establish the "Good Faith" and "Public Interest" nature of my disclosures, findings which are necessary to discharge the Rule Nisi and prove that the Applicant approaches this Court with "dirty hands."

2. THE RESPONDENT'S BONA FIDES, TRANSPARENCY, AND CLINICAL AUTHORITY

- 2.1 The Applicant's entire application rests on the false premise that I am a vexatious, bad-faith actor. I respectfully direct this Honourable Court to the documentary evidence of

my character, which proves a consistent pattern of radical transparency and patient advocacy.

2.2 Radical Transparency: Unlike the Applicant, who relies on institutional silence and altered documents, my conduct has always been entirely transparent. In January 2024, during my recruitment, I proactively disclosed a pending legal/medical matter to the Applicant's Human Capital department (**Annexure [Vacancy Email]**). I did not hide my vulnerabilities. My transparency is a matter of record from before my employment even began.

2.3 Clinical Authority and Patient Advocacy: The Applicant attempts to frame my ethical refusal to blame a patient for a system error as "insubordination." As evidenced by the reference from Dr. P.M.J. Ohm of the Mitchells Plain District Hospital Emergency Unit (**Annexure RESU1**), my career is defined by a "depth of clinical knowledge" and a commitment to "protect patient dignity." My actions in October 2025 were the exact high-level patient advocacy I am known for.

2.4 The Title "Bait-and-Switch": I was actively recruited and interviewed specifically for the role of "**Specialist Care Manager**," as explicitly stated in the Applicant's own interview invitations. It is highly deceitful that the Applicant recruited me as a Specialist, recognized me internally as a Case Manager, yet is now attempting to downgrade my title on statutory exit documents to cover up my authority to identify a systemic integration gap.

3. THE GENESIS: ETHICAL REFUSAL AND PATIENT ADVOCACY (OCTOBER 2025)

3.1 In October 2025, I conducted a Root Cause Analysis (RCA) on Case [GEMS 000655789] (**Annexure VICT1**). I identified a "documented system failure" between the Gexus and DMS platforms. I was instructed by management to contact the affected member and address "non-adherence," effectively misattributing a corporate system fault to a patient.

3.2 Bound by the ethical codes of the Nursing Act, I refused to misattribute fault. In a recorded meeting (**Annexure FG2 / IMP1**), my team leader characterized this ethical refusal as "insubordination."

4. THE PUBLIC INTEREST: APN 303 AND THE R3 BILLION "GOVERNANCE TAX"

- 4.1** At Paragraph 40 of their Founding Affidavit, the Applicant admits the Gexus and DMS platforms manage chronic care pathways and adherence data. Actuaries rely entirely on Gexus/DMS data to build statutory financial models under **Actuarial Practice Note 303 (APN 303)**.
- 4.2** As proven in my RCA, the systemic integration gap generates "false non-adherence" data. When the system fails to recognize that members are compliant, actuaries are forced to apply maximum APN 303 **"Risk Loading"** to compensate for the data vacuum.
- 4.3** The Council for Medical Schemes (CMS) stipulates a 3.3% to 4% increase for inflation. The scheme, despite a reported R2 billion deficit, is levying an exorbitant **9.5% premium increase**.

4.4 The excessive 4.5% difference is not medical inflation; it is an artificial **Governance Tax** caused by the system "flying blind." Had the Applicant remediated the integration gap I identified, the system would accurately recognize adherence, eliminating the APN 303 risk loading and saving the scheme's 1.9 million members an estimated **R3.016 billion**.

5. THE TIMELINE OF SABOTAGE AND PSYCHOLOGICAL WARFARE

- 5.1** The Applicant's Founding Affidavit deliberately isolates the events of December 2025 to hide a historical machinery of retaliation.
- 5.2 Dignity Violations:** By mid-2025, the workplace was already rendered hostile. I was forced to file formal grievances regarding workplace discrimination (**Annexure FG1**). I was mocked in team-wide communications (**Annexure ONE1**) and my dignity was continuously undermined (**Annexures SCR1 and DER1**).
- 5.3 Operational Sabotage:** My line manager actively interfered with my clinical duties (**Annexure M1**) by deliberately assigning me duplicate "LACE cases" already allocated to other staff, forcing me to waste hours opening redundant files.
- 5.4 The First Financial Strangling (July 2025):** On Payday, **23 July 2025**, management executed a punitive, unconsulted deduction of **40.47% (R13,824.32)** of my normal earnings, explicitly refusing to stagger this debt (**Annexure LIE1**). The very next day, **24 July 2025**, management sent an unprompted policy document (**Annexure POL1**). This timing was calculated psychological warfare.

5.5 Medical Weaponization: When I experienced severe, documented medical distress requiring urgent medication, management weaponized the absence management policy (**Annexures BDC1, MV1, AMR1**).

5.6 Malicious Disregard for Mental Health: The Applicant's malice is further documented in their CCMA Opposing Affidavit, where the deponent — who has never personally met me — swore under oath that my documented PTSD and depression were mere "excuses" and an "afterthought."

6. INSTITUTIONAL SILENCE AND THE CEO'S BREACH

6.1 When I uncovered the R3 billion integration gap in October 2025, the Applicant met my internal reports with **30 days of absolute institutional silence**.

6.2 Having exhausted internal remedies, I submitted an Executive Leadership Escalation to the Group CEO on **4 December 2025**. Instead of applying whistleblower protections, the CEO personally breached my confidentiality by referring my disclosure back to the very managers orchestrating the victimization (**Annexure DEC1**).

6.3 The CEO's Perjury Regarding Constructive Dismissal: The Group CEO deposes under oath in his Founding Affidavit that I "allegedly resigned." As proven in **Annexure RES1**, on **12 December 2025**, I personally served a 4-page formal notice titled "*Notice of Resignation Due to Automatically Unfair Constructive Dismissal*" directly to the CEO's email address.

7. POST-RESIGNATION DETRIMENT AND STATUTORY FRAUD

7.1 The retaliation did not cease upon my forced exit. In January 2026, the Applicant systematically obstructed my access to statutory UI-19 unemployment benefits and retirement funds (**Annexures EXIT1, EXIT2**).

7.2 Statutory Misrepresentation: When the Applicant finally produced a Certificate of Service (**Annexure EXIT3**), they committed a gross statutory misrepresentation, deliberately downgrading my role from **Specialist Nurse / Case Manager** to align with the fabricated narrative in their CCMA Opposing Affidavit.

7.3 The Administrative Deadlock and Severe Prejudice: The Applicant's fraudulent alteration of my UI-19 has created a devastating administrative deadlock. As evidenced by correspondence from the Department of Employment and Labour dated **15 April 2026** (**Annexure [DEL Email]**), the Department has refused to investigate the statutory fraud

or unblock my UIF benefits until the CCMA formally confirms my Constructive Dismissal.

7.4 I am trapped in a state of ongoing, severe prejudice. The Applicant has financially strangled me, and the regulatory bodies are paralyzed. I submit that this Honourable Court is the *only forum* with the inherent jurisdiction capable of breaking this deadlock and stopping the immediate financial and psychological bleeding.

8. THE FATAL CONTRADICTION IN THE APPLICANT'S PAPERS

8.1 The Applicant's bad faith is unequivocally proven by their own urgent High Court application. Throughout the internal correspondence attached as Annexures to the Group CEO's Founding Affidavit, my professional signature and system designation are repeatedly and undisputedly recorded as **"Case Manager: GEMS Active Disease Risk Management."**

8.2 They cannot rely on my internal communications as a "Case Manager" to secure a High Court interdict, while simultaneously erasing that same title on Department of Labour documents to diminish my forensic credibility.

9. STATUTORY DUTY TO UNIONS AND REGULATORY BODIES

9.1 Paragraph 2.1 of the Interim Order interdicts me from communicating with the PSA, NEHAWU, DENOSA, and SAPS.

9.2 As a Specialist Nurse, I have a fiduciary duty to the scheme's 1.9 million members. Given that the 9.5% premium increase is an artificial Governance Tax directly harming public servants, my intention was to fulfill a statutory healthcare mandate.

10. DEMAND FOR DISCOVERY

10.1 I formally challenge the Applicant to produce, prior to the return date, the following:

- (i) The unredacted CMS Business Plan justifying the 9.5% premium increase;
- (ii) The Actuarial Risk Profiling Logs fed from the Gexus/DMS platforms into the APN 303 models since October 2025; and
- (iii) The unedited Microsoft Teams recording titled "Naming Convention."

11. PRAYER FOR DECLARATORY RELIEF

Wherefore, I respectfully pray that this Honourable Court discharges the Rule Nisi with a punitive costs order against the Applicant, and makes the following declaratory findings:

1. That the Applicant's conduct from mid-2025 constituted a sustained campaign of unlawful workplace victimization;
2. That the Root Cause Analysis (Annexure VICT1) and subsequent escalations regarding the Gexus/DMS integration gap constitute Protected Disclosures made in the public interest;
3. That this timeline of escalating retaliation engineered a state of Administrative and Professional Impossibility; and
4. That the Respondent was unlawfully ousted, and the employment environment was rendered objectively intolerable per se, thereby confirming constructive dismissal. This declaration is urgently sought not only to defeat the Applicant's claim to "clean hands," but to immediately break the administrative deadlock with the Department of Employment and Labour, allowing the Respondent access to unlawfully withheld statutory social security benefits.

DEPONENT **SR. WILBUR W. MATTHEE**

Specialist Registered Nurse (NQF Level 8) | Forensic Governance Specialist

COMMISSIONER OF OATHS

Signed and sworn before me at **CAPE TOWN** on this _____ day of _____
2026.

The deponent has acknowledged that he:

- (a) knows and understands the contents of this affidavit;
- (b) has no objection to taking the prescribed oath; and
- (c) considers the oath to be binding on his conscience.

Signature of Deponent	Signature and Stamp of Commissioner of Oaths
Full Name of Commissioner	Capacity and Area of Jurisdiction

EthicHawks Forensic Governance & Compliance Consultancy | info@ethichawks.co.za | Cape Town, South Africa